

Certainly! Here is the summary of the **Diagnosis, Assessment, Treatment, and Exacerbations** sections from the **GOLD 2026 Report** in English:

1. Diagnosis

- **When to suspect COPD:** A diagnosis should be considered in any patient who has **dyspnea** (progressive, persistent, or worse with exercise), **chronic cough** (may be intermittent or non-productive), **sputum production**, or a history of recurrent lower respiratory tract infections and/or exposure to risk factors (tobacco smoke, pollution, occupational hazards, or host factors) 1, 2.
- **The Gold Standard:** Forced **spirometry** is mandatory to establish the diagnosis 1, 3, 4.
- **Confirmation:** The presence of a **post-bronchodilator FEV1/FVC < 0.7** confirms persistent airflow obstruction and the diagnosis of COPD 1, 5, 6.

2. Initial Assessment

Following confirmation, the assessment focuses on five fundamental aspects to guide therapy:

- **Severity of Airflow Obstruction:** Classified into **GOLD 1 (Mild) to GOLD 4 (Very Severe)** based on the post-bronchodilator FEV1 % predicted 7.
- **Symptoms:** Formally assessed using validated tools like the **mMRC** (dyspnea scale) and the **CAAT™/CAT™** (comprehensive health status assessment) 7-9.
- **Exacerbation History:** Identifying the risk based on the frequency and severity of events in the previous year 10-12.
- **Blood Eosinophil Count:** Used as a biomarker to guide the use of inhaled corticosteroids (ICS) for exacerbation prevention 10, 13, 14.
- **Multimorbidity:** Actively searching for and treating comorbid conditions (e.g., cardiovascular disease, lung cancer, osteoporosis, anxiety/depression) as they independently influence health status and prognosis 7, 14, 15.
- **ABE Classification:** Patients are assigned to Group A, B, or E based on their symptoms and exacerbation history to determine initial treatment 12, 16, 17.

3. Management and Treatment

The management cycle involves a continuous process of **Review, Assess, and Adjust** 18, 19.

- **Initial Pharmacological Treatment:**
- **Group A:** A single bronchodilator (short or long-acting) 20.
- **Group B:** Dual long-acting bronchodilation (**LABA + LAMA**) 21.
- **Group E:** Dual bronchodilation (**LABA + LAMA**); consider triple therapy (**LABA + LAMA + ICS**) if blood eosinophils are ≥ 300 cells/ μ L 22, 23.
- **Follow-up Treatment:** Adjusted based on the patient's predominant "treatable trait": either **persistent dyspnea** or **occurrence of exacerbations** 24, 25.
- **Non-Pharmacological Treatment:** Essential measures include **smoking cessation**, **vaccinations** (Influenza, COVID-19, Pneumococcal, RSV, Tdap, and Shingles), and **pulmonary rehabilitation** 26-30.

4. Exacerbations

- **Definition:** An acute event characterized by a worsening of respiratory symptoms over a period of up to 14 days, often associated with increased local and systemic inflammation 31, 32.
- **Severity Classification (Rome Proposal):** Severity is now classified at the point of care based on clinical variables:
- **Mild:** Low symptom intensity (Dyspnea VAS < 5) and stable vital signs 33, 34.
- **Moderate:** Requires treatment with systemic corticosteroids and/or antibiotics 33, 34.
- **Severe:** Requires hospitalization and may involve acute respiratory failure 33, 35.
- **Management:**
- **Bronchodilators:** Short-acting beta2-agonists (**SABA**), with or without short-acting anticholinergics 36, 37.
- **Corticosteroids:** Systemic steroids (e.g., 40 mg prednisone) for **5 days** 36, 38.
- **Antibiotics:** Indicated if signs of bacterial infection (e.g., purulent sputum) are present, typically for **5 days** 36, 39, 40.

Does this English version meet your needs, or would you like more details on a specific section?